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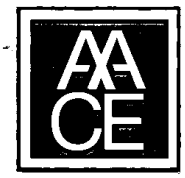

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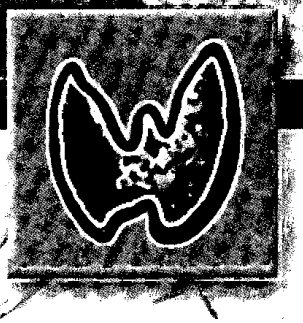
Information for Patients

Thyroid Carcinoma

Prepared by the American Association of Clinical Endocrinologists (AACE), a not-for-profit national organization of specially qualified board-certified endocrinologists whose aims include maintaining the high standards of its physician members and providing high-quality care to patients with endocrine disorders.



What is thyroid carcinoma?

Thyroid carcinoma (cancer) is a rare condition that affects the thyroid gland. It usually appears as a lump or bump in the front of the neck, below the larynx ("Adam's apple") and above the collarbone (the site of the thyroid). In most cases, the lump is painless and affects one side, and the results of thyroid function tests (blood tests) are entirely normal. However, any unexplained growth should be investigated by your physician.

After that, the appropriate "suppressive" dose of thyroid medication (levothyroxine) should be given to decrease the likelihood of recurrence. Careful monitoring for any signs of cancer recurrence is supervised by the endocrinologist and may include ultrasound examinations, scans, and periodic testing of a blood measurement called thyroglobulin.

The need for further monitoring studies to be certain that the cancer does not recur will be determined by your physician. Fortunately, most types of thyroid cancer are associated with a very good prognosis when diagnosed early and treated by a physician who is familiar with the management of this disease.

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How is thyroid cancer treated?

Fortunately, most types of thyroid cancer can be diagnosed early and cured completely, but a thoughtful and comprehensive investigation is necessary. If thyroid cancer is suspected after review of all the information, then referral to an experienced thyroid surgeon is recommended.

The usual approach is to remove the entire lump, the portion of the thyroid containing the lump, and often most of the remaining thyroid gland. If the cancer is confirmed after surgery, further consultation with the endocrinologist should be considered. Radioactive iodine treatment may be recommended in order to destroy any remaining malignant thyroid cells and to reduce the risk of recurrence of this disease.

What are the symptoms of thyroid cancer?

Most patients with thyroid cancer have a gradually enlarging lump within the thyroid gland (located in the front portion of the neck). It usually moves with swallowing. Occasionally, the lump may cause a feeling of pressure, but usually no symptoms are present except the fullness (lump) in the neck. Obviously, finding a lump or bump in the neck should be brought to your physician's attention, even in the absence of symptoms.

What are the causes of thyroid cancer?

As with many types of cancer, the specific reason for developing thyroid carcinoma may remain a mystery. For your assessment, several risk factors will be evaluated by your physician:

- Radiation to the head or neck, especially during childhood
- Genetic predisposition (the influence of heredity)
- Gender (men are at somewhat higher risk for a cancerous lump in the thyroid than are women)

How is thyroid cancer diagnosed?

First, your physician takes a careful and detailed history and performs a physical examination, especially of the thyroid gland. The tests available to your physician include, but are not limited to, the following:

- Blood studies
- Thyroid uptake (absorption) and scan
- Ultrasonography of the thyroid
- Fine-needle aspiration biopsy of the thyroid

The tests best suited for a specific patient are determined by your physician after careful consideration of all the facts. If you are concerned about the potential presence of thyroid cancer, consultation with an endocrinologist is appropriate.